DECISION MEMO ONTARIO PATIENT TRANSPORTATION AUGUST 2026

Build, operate, or neither

Two questions were put to the Phase 0 research: is there a market for this software, and what shape should it take. This memo answers both, and names the one action that makes the decision for you.

THE VERDICT

Software alone, sold as software, into Ontario patient transportation is a weak business. The problem is enormous and real. The willingness to pay for software specifically is unproven, and the addressable pool does not justify the clinical and regulatory work the domain demands.

The pain and the market are different claims, and conflating them is how this fails. What follows separates them, then names the one shape that works.

01 The pain is not in doubt

Four in five inter-facility transfers in Ontario are clinically routine, yet a large share still ride 911 ambulances inside a system spending on the order of **$2B a year**. ALC patients hold roughly one bed in five at The Ottawa Hospital's Civic campus. One dialysis patient in ten misses a session every month, with transportation a contributing cause. Five separate networks move the same people on different days and none can see the others.

None of that is disputed, and none of it is sufficient. **A market needs someone with a budget who will sign.**

02 Three findings that say be careful

FINDING 01

There is no payer

OHIP does not cover non-emergency transport. No broker layer, no claims rail — the two things every US NEMT platform plugs into. **You cannot import that architecture because there is nothing to import it into.**

FINDING 02

The SaaS ceiling is low

Per-vehicle dispatch software tops out at **$3.4–5.8M ARR** against every eligible vehicle in Ontario. At realistic penetration, under $2M. That is a lifestyle business carrying a clinical compliance burden.

FINDING 03

Buyers don't buy on architecture

Prehos collapsed *commercially* in 2026 and displaced ~22 Ontario services — prepaid fees stranded, hardware forced. They migrated **cloud to cloud**, taking two vendors to replace one. Burned by vendor risk, they still chose another backend platform.

Finding 03 is the natural experiment, and it should change the pitch

It is the closest thing this market has to a controlled test of whether buyers reward better architecture under duress. **They did not.** They rewarded compliance, references and support.

Sell the outcome, not the design. Data ownership and continuity are a strong second argument and a poor first one.

03 What is actually monetizable

Narrower than "patient transportation software" — and each is a contract, not a seat.

OPPORTUNITY	WHY IT'S REAL	SHAPE
Dialysis transport Regional renal programs	~109,000 Ottawa round trips a year under **one clinical owner**, against an outcome — missed treatments — the buyer already reports on	Service contract
NIHB medical transport Indigenous Services Canada	**$166M in Ontario, growing ~20% a year.** A single federal payer already contracting — the cleanest payer structure in the market	Payer utility
Discharge and ALC flow Hospitals	Hospitals are measured on bed-days and will pay against them. Transport is the last-mile blocker a vendor can actually move	Outcome share

---

What the research found was the **spend**. What it did not find was a **budget owner** — a line item someone already holds and is dissatisfied with. That gap is the difference between a strong yes and a qualified one, and it is answerable in a week of conversations.

04 The format

Not a product format at all.

RECOMMENDED SHAPE

Operate the Ottawa dialysis book. Instrument it. Sell the coordination layer outward from a working reference site.

1. **It sidesteps the missing payer.** You are paid for a service someone already buys, not for software nobody holds a budget line for.
2. **It generates the data that makes a platform credible** — scheduling, utilisation, missed-treatment outcomes. That layer cannot be cold-sold without it.
3. **It puts you inside the buyer relationship** rather than pitching into it from outside.
4. **It exercises the EMSS core in production.** The Ontario Provincial Client Registry FHIR adapter, the op-log sync engine and the PHIPA primitives stop being assets on paper.
5. **It produces the reference case** that both the Toronto and NIHB conversations require before they can start.

Operate → Instrument → Coordination layer → Settlement rail

WHY NOT PURE SAAS

- A **$3.4–5.8M** ceiling at total penetration
- Nothing to plug into — no broker, no claims rail
- Competing with US incumbents who have better dispatch products and nothing to do with them here
- Consolidators are installing captive portals at contract hospitals every year you wait

WHY NOT PURE OPERATOR

- Low margin, capital-intensive, **labour is the binding constraint**
- Break-even needs 7 vehicles at 85% utilisation — **13 at 72%**
- Entering against a global consolidator that can price below cost longer than you can
- No compounding asset if you stop at vehicles

The hybrid is the only shape where each half fixes the other's weakness. Operating solves software's payer problem; the software layer solves operating's margin and defensibility problem.

Longer term the defensible position is the **eligibility and settlement rail** — determining who pays which fraction across NIHB, the Northern grant, hospital, LTC and patient. It genuinely does not exist and the fragmentation is permanent. But it is the slowest build, and it needs the beachhead first.

05 The case against

This deserves stating as plainly as the recommendation, because the research supports it just as readily.

If the constraint is capital and speed, don't do this

The EMSS product's own commercialization analysis scores official health-system selling as a fortress and recommends the humanitarian path instead: **revenue in months, light regulatory load**, and it plays to the no-backend differentiator that transport actively contradicts.

Ontario patient transportation is a **three-to-five-year build against a fragmented buyer**, and a second front for the same team. Saying no is a legitimate answer, not a failure of nerve.

What would move this to a strong yes

Evidence that a hospital or renal program **already holds a line item for transport they are dissatisfied with.** The research located the spend; it did not locate the owner. That single fact separates "there is a problem" from "there is a buyer," and no amount of further desk research will settle it.

06 The next action

DO THIS BEFORE ANYTHING ELSE

One conversation with The Ottawa Hospital's Regional Nephrology Program

Three questions: what missed treatments cost them in cancelled slots, what they believe transport costs them today, and whether a contracted pooled service would interest them.

That conversation costs nothing and collapses the entire decision tree. With a yes, the fleet fills from day one, utilisation risk mostly disappears, and the platform has a reference site. With a no, you are a seventh entrant in a $2–8M market against a global operator, and the answer is no.

Everything else in the Phase 0 file — the survey, the opportunity model, the business case — is a plan waiting on it.

Basis. This memo synthesises the Phase 0 file and adds no new research. Figures come from **docs/01-market-survey.md**, **docs/02-opportunity-model.md** and **docs/05-ottawa-business-case.md**, and carry the confidence ratings recorded in **docs/03-source-register.md**.

The caveat that governs all of it. Network restrictions during research blocked direct retrieval of primary sources, so most figures were reconstructed from search-result summaries rather than the documents themselves, and six of nine cost inputs in the business case are modelled rather than quoted. That is adequate for deciding direction. It is not adequate for a funding application, and the verification list says what to fix first.